

Stroke Coordinator

Comprehensive Stroke Center (CSC)

Aligned with the 2026 AHA/ASA Guideline for Early Management of Acute Ischemic Stroke and AHA Quality Initiatives

Position Summary

Coordinates and oversees the hospital's Comprehensive Stroke Center (CSC) program, ensuring care delivery meets current American Heart Association/American Stroke Association (AHA/ASA) evidence-based benchmarks — including the 2026 guideline update — for acute ischemic stroke (AIS), hemorrhagic stroke, and endovascular therapy (EVT), and that the program performs to Target: Stroke and Get With The Guidelines–Stroke (GWTG-Stroke) standards. A full glossary of abbreviations used throughout this description appears at the end of the document.

Key Responsibilities

Program Management

- Maintain protocols aligned with the 2026 Guideline for the Early Management of Patients With AIS, which replaces the 2018 guideline and its 2019 update.
- Lead the transition of stroke protocols to reflect 2026 practice changes, including:
 - **Tenecteplase:** add as an approved thrombolytic option alongside alteplase within the 4.5-hour window, based on trial evidence showing noninferiority and possible advantages over alteplase.
 - **EVT eligibility:** update criteria to capture patients with basilar artery occlusion and large ischemic cores who were previously excluded from intervention.
 - **Extended-window thrombolysis:** build a pathway for the 4.5–9 hour window in select patients based on advanced imaging, and prioritize treatment for disabling deficits regardless of NIHSS score.
 - **EMS bypass protocols:** review against updated guidance to transport to the closest EVT-capable hospital when rapid interhospital transfer systems aren't well established.
 - **Mobile stroke units:** evaluate feasibility of integration, now endorsed in the 2026 guideline, if applicable to the region's stroke system.
 - **Pediatric AIS pathway:** establish a pathway addressing the guideline's first-ever pediatric stroke recommendations.
 - **Hyperglycemia and dysphagia management:** update protocols per new evidence incorporated in the 2026 update.

Clinical Coordination — AHA/ASA Time Targets

- **Door-to-needle (DTN):** within 60 minutes in 85% or more of patients treated with IV thrombolytics, with secondary goals of 45 minutes in 75% of patients and 30 minutes in 50%.
- **Door-to-device (thrombectomy):** within 90 minutes for direct-arriving patients.
- **Door-to-groin puncture:** commonly tracked at 90 minutes or less.
- **Door-in-door-out (DIDO) for transfers:** ≤90 minutes in 50% or more of transferred patients.
- Implement AHA best-practice bundles: EMS prenotification, single-call stroke team activation, rapid imaging acquisition/interpretation, and standardized protocols.
- Screen incoming and in-house patients using updated large-core and basilar occlusion EVT criteria; ensure imaging protocols support extended-window thrombolysis decision-making.

Data & Quality Improvement

- Abstract and submit data to GWTG-Stroke; track tenecteplase vs. alteplase utilization and outcomes as the protocol transitions.
- Monitor EVT volumes and outcomes for the newly eligible large-core and basilar occlusion populations, since these represent an expanded case mix.
- Present GWTG-Stroke and Target: Stroke performance data, plus 2026-guideline implementation progress, to the stroke committee and hospital leadership.

Education & Outreach

- Retrain ED, ICU, neurology, and neurointerventional staff on 2026 guideline changes — particularly the new thrombolytic option, expanded EVT criteria, and extended treatment windows.
- Update EMS partner training on triage/transport protocols reflecting the "closest EVT-capable hospital" transfer guidance.
- If pursuing pediatric stroke capability, coordinate education for ED and pediatric staff on the new pathway.

Regulatory/Survey Readiness

- Maintain documentation showing protocols, order sets, and competencies have been updated to reflect the 2026 guideline (surveyors will expect current-guideline alignment).
- Track and document sustained achievement of AHA time targets under the revised protocols.

Qualifications

- **Education:** BSN required; MSN often preferred.
- **Experience:** 5+ years in neuroscience/neuro ICU or high-acuity stroke care.
- **Certifications:** SCRNP strongly preferred, NIHSS certification, ACLS.
- **Skills:** GWTG-Stroke registry/data abstraction, familiarity with AHA quality-improvement toolkits, experience leading protocol/order-set revisions tied to guideline updates.

Glossary of Abbreviations

- **AHA** — American Heart Association
- **ASA** — American Stroke Association
- **CSC** — Comprehensive Stroke Center
- **PSC** — Primary Stroke Center
- **AIS** — Acute Ischemic Stroke
- **EVT** — Endovascular Therapy (mechanical thrombectomy)
- **IV tPA** — Intravenous Tissue Plasminogen Activator (a clot-dissolving thrombolytic medication)
- **DTN** — Door-to-Needle time (arrival to start of IV thrombolytic administration)
- **DIDO** — Door-in-Door-out time (arrival to departure for patients being transferred to a higher level of care)
- **NIHSS** — National Institutes of Health Stroke Scale (a standardized tool for scoring stroke severity)
- **GWTG-Stroke** — Get With The Guidelines–Stroke (AHA's national stroke data registry and quality program)
- **SCRNP** — Stroke Certified Registered Nurse
- **CNRN** — Certified Neuroscience Registered Nurse
- **ACLS** — Advanced Cardiovascular Life Support (certification)

- **BSN** — Bachelor of Science in Nursing
- **MSN** — Master of Science in Nursing
- **RN** — Registered Nurse
- **ED** — Emergency Department
- **ICU** — Intensive Care Unit
- **EMS** — Emergency Medical Services
- **PI** — Performance Improvement
- **mRS** — modified Rankin Scale (a scale measuring degree of disability/dependence after stroke)
- **LVO** — Large Vessel Occlusion
- **SAH** — Subarachnoid Hemorrhage